YEAR 9–10 · EVIDENCE BRIEF · OCTOBER 2026

Prevention Strategies & Priority Populations

Real research findings for students to evaluate, drawn from Life Saving Victoria's Aquatic Injury Prevention Agenda 2026–27. Built for the part of the curriculum that asks students to critique strategies and health information — not just recall safety rules.

VC2HP10P08 | VC2HP10P09 | VC2HP10P10 | 6 tasks | Victorian data

How to use this brief

Each section gives students a real finding from published research or Victorian surveillance data, then asks them to do something with it. The findings are deliberately uncomfortable: several of them undercut assumptions the rest of this unit teaches. That is the point — **VC2HP10P09 asks students to critique health information and messaging, which they cannot do if every source they meet agrees with every other source.**

Curriculum codes are shown per section. All data is Victorian unless stated. Source references are listed in full at the end.

1. Who the priority groups actually are

Life Saving Victoria sets prevention priorities annually and grades them by urgency. For 2026–27, six areas are rated **urgent**: coastal waterways; boating and watercraft; young people aged 15–24; high-risk populations; males aged 45–64; and disaster and extreme weather.

Two of those six are the cohorts this unit is written for or about.

YOUNG PEOPLE AGED 15–24 · FIVE-YEAR AVERAGE

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5

fatal drownings each year on average

81%

were male

50%

occurred on weekends or public holidays

65%

were swimming, paddling or wading

46%

involved people from multicultural backgrounds

That last figure is the one worth sitting with. Across all ages in Victoria, people from multicultural backgrounds account for 37% of drowning deaths since 2015–16. Among 15–24-year-olds specifically, it is **46%** — nine percentage points higher, in the age group closest to the students reading this.

Task 1.1 — Reading a disaggregated figure `VC2HP10P09`

1. The all-ages figure is 37%. The 15–24 figure is 46%. Write one sentence stating what the difference tells you, and one sentence stating what it does *not* tell you.
2. Life Saving Victoria records country of birth as unknown in 22% of Victorian drowning deaths. Explain, in your own words, why both figures should be described as lower bounds rather than exact percentages.
3. A newspaper reports the 46% figure under the headline "Migrant teens drowning at double the rate". Identify two things wrong with that headline.

2. The finding that undercuts "swim between the flags"

Every resource in this unit teaches the red-and-yellow flags as the core safety message. A 2026 study tested whether the symbol actually communicates that.

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Researchers surveyed 163 South Korean university students with mixed English proficiency about how they would behave on Australian beaches and how they read beach safety signage. Most had never visited Australia.

86% misinterpreted the 'swim between the flags' symbol. It was read as marking a danger zone, a sports event area, or a private zone — rather than the safest place on the beach to swim. Terms including 'high surf', 'submerged objects', 'shore dump' and 'rip currents' were not understood by most respondents.

A second finding: 91% said they would not enter the water if a 'dangerous currents' sign was displayed. But when the same students were given a scenario in which other people were already in the water, they became more likely to enter regardless of the sign.

Shibata et al. (2026b), reported in LSV Aquatic Injury Prevention Agenda 2026–27, p.42

Why this matters for how we explain over-representation

There are two ways to explain why people from multicultural backgrounds are over-represented in drowning statistics. The first locates the problem in the people: they did not learn to swim, they do not know the rules. The second locates it in the system: the safety information was designed for people who already understood it.

This study supports the second explanation. A symbol that 86% of a surveyed group reads as meaning the *opposite* of what it means is not a failure of the audience. **Students should be able to tell the difference between a deficit explanation and a structural one, and to recognise which one the evidence supports.**

Task 2.1 — Redesigning the message `VC2HP10P08` `VC2HP10P10`

1. The flags symbol relies on prior knowledge to be understood. Design a replacement beach sign that would communicate "this is the safest area to swim" to someone who has never been to an Australian beach and reads limited English. Sketch it and annotate your choices.
2. The second finding — people entering the water despite a warning sign because others were already in it — is called **social proof**. Explain why a sign alone cannot overcome social proof, and name one intervention that could.

3. LSV's recommendation for coastal waterways is to "facilitate access to multilingual coastal safety education and signage". Based on this research, argue whether translation alone would be sufficient.

3. Why knowing the risk does not change the behaviour

A persistent puzzle in water safety is that the people most at risk often already know the information. A 2025 Australian study examined this directly.

> Twenty-three Australian males aged 18–30 who had previously drunk alcohol around water were interviewed about their decisions. Drinking in aquatic settings was typically social, spontaneous and unplanned, often at beaches, pools and rivers without lifeguards.
>
> Participants **acknowledged that drinking increased drowning risk**. Many nonetheless believed they could accurately judge their own level of intoxication, the water conditions and their swimming ability. Risk was frequently considered only after the event, and for some participants, not at all.
>
> Social identity shaped behaviour strongly. Participants described the 'typical' person who drinks and swims as young, male, adventurous, sensation-seeking and reckless — and viewed that as socially undesirable. They then described *their own* similar behaviour as controlled and sensible. Peer groups normalised the risk even where participants recognised broader social disapproval.
>
> *Smith et al. (2025), reported in LSV Aquatic Injury Prevention Agenda 2026–27, p.36–37*

The researchers name the mechanisms: **optimism bias** (the belief that bad outcomes happen to other people), masculine norms, overconfidence in swimming ability, and peer dynamics. Their conclusion is that awareness campaigns which simply restate the risk are addressing a problem that does not exist — the awareness is already there.

Task 3.1 — The knowing–doing gap `VC2HP10P06` `VC2HP10P09`

1. Participants described others who drink and swim as reckless, while describing their own identical behaviour as sensible. Name this pattern and explain what it means for campaigns built on "don't be reckless" messaging.
2. LSV's recommendation for this cohort is to "promote responsible mateship". Explain how that recommendation responds to the peer-dynamics finding, rather than to the awareness finding.

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3. Write two campaign slogans: one that targets awareness, and one that targets peer responsibility. Explain which the evidence supports and why.

4. Does a social media campaign work?

> Queensland Parks and Wildlife Service and researchers co-designed an Instagram campaign communicating aquatic safety risk at a popular national park site, informed by interviews with travel influencers and a survey of 509 social media users. Instagram was identified as the most influential channel for trip planning among 18–35-year-olds.
>
> Over January–February 2024 the campaign reached more than 960,000 users, generating 4,000 link clicks, 254 post saves and over 100 shares. On-site surveys found a third of visitors said social media had influenced their decision to visit. Of those surveyed, 32% recognised at least one campaign post and 20% found the messages useful.
>
> **20% viewed the tone as condescending or passive-aggressive.**
>
> *Cornell et al. (2025), reported in LSV Aquatic Injury Prevention Agenda 2026–27, p.37*

Task 4.1 — Evaluating reach against impact `VC2HP10P09` `VC2HP10P10`

1. The campaign reached 960,000 users. 20% of those surveyed found the messages useful. Explain why reach and impact are different measures, and which one a funder should care about.
2. Equal numbers found the campaign useful (20%) and found its tone condescending (20%). What does that tell you about designing safety messaging for your own age group?
3. Design one Instagram post for 15–24-year-olds about inland waterway safety. Then write 50 words justifying your tone choice using this study as evidence.

5. A prevention success worth studying

Most of this unit deals with problems. One Victorian figure runs the other way: fatal drownings involving alcohol and/or drugs have fallen **67%** against the Victorian Water Safety Strategy baseline.

That decline did not come from a single intervention. It followed sustained work across legislation, enforcement, public campaigns and changing social norms around drinking — the same combination that drove the 63% reduction in drowning among children aged 0–4 following pool fencing legislation in Australia.

Task 5.1 — What makes prevention work `VC2HP10P08` `VC2HP10P10`

1. Pool fencing (63% reduction in 0–4 drowning) and alcohol-related drowning (67% reduction) are the two clearest Australian prevention successes. Identify what the two have in common that education-only campaigns do not.
2. LSV recommends restricting "alcohol availability and advertising around aquatic environments". Build the strongest case *for* that policy, then the strongest case *against*, then state your own position.
3. Using the priority list in Section 1, choose one urgent priority not covered in this brief and propose a prevention strategy for it. Justify the strategy against evidence, cost and the population it targets.

6. Reading surveillance data honestly

A trap built into this report

The Aquatic Injury Prevention Agenda reports **35 fatal drownings in Victoria**. The Victorian Drowning Report reports **52**. Both are current. Both are from Life Saving Victoria.

They are not in conflict. The 35 covers **1 July 2025 to 28 February 2026 — eight months**, not a year. The 52 covers the full 2024–25 financial year. Comparing them directly would produce a 33% "decline" that does not exist.

Against the correct comparison — the five-year average *for the same eight-month period* — the 35 is a **14% decrease**.

Task 6.1 — Comparing like with like `VC2HP10P09`

1. Explain in your own words why 35 and 52 cannot be compared, even though both are Victorian drowning figures published by the same organisation.

2. A student writes: "Victorian drownings fell from 52 to 35, a drop of 33%." Rewrite the sentence so it is accurate.
3. The report also states 18 people drowned over summer 2025–26, a 20% decrease on the five-year average for that period. Explain why the report compares each period against its own average rather than against the previous full year.

Teacher notes

Sequencing. Sections 1, 2 and 6 work as a block on data literacy and health equity. Sections 3 and 4 work as a block on behaviour change and media. Section 5 is the natural lead-in to the assessment task in the Year 9–10 workbook.

Handle Section 2 carefully. The signage research is about how well Australian safety symbols communicate, not about the competence of any community. Frame it as a design failure throughout. If the discussion starts drifting toward deficit language about migrant communities, redirect to the question: who was the sign designed for?

Section 3 touches alcohol. Alcohol and other drugs is one of the twelve mandated focus areas, and this material is about risk perception rather than substance use itself. Follow your school's position on AOD content and check the Positive Choices guidance referenced in the HPE Resource Directory.

Data currency. The Aquatic Injury Prevention Agenda is an interim report. Life Saving Victoria's full Victorian Drowning Report 2025–26 was not published when this brief was written; when it appears, Section 6's figures should be rechecked against it.

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Sources

Life Saving Victoria. (2026). *Aquatic Injury Prevention Agenda 2026–27*. Life Saving Victoria: Melbourne. — all Victorian surveillance figures, priority ratings and prevention recommendations in this brief.

Life Saving Victoria. (2025). *Victorian Drowning Report 2024–25*. Life Saving Victoria: Melbourne. — the 52 full-year figure and the 37% all-ages multicultural figure.

Research summarised within the Aquatic Injury Prevention Agenda 2026–27: Shibata et al. (2026b) on beach signage comprehension (p.42); Smith et al. (2025) on young males, alcohol and risk perception (p.36–37); Cornell et al. (2025) on social media safety campaigns (p.37).

Research summaries in this brief are paraphrased from Life Saving Victoria's own summaries of those studies. Students citing them in assessed work should cite the Aquatic Injury Prevention Agenda as the source, or locate the original papers.

← Back to the Swim & Water Safety Teaching Hub

Victorian Curriculum F–10 Version 2.0 · Health and Physical Education · Levels 9–10
Prevention priorities and research summaries from Life Saving Victoria's Aquatic Injury Prevention Agenda 2026–27.

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Data sources cited in this resource. Drowning statistics are reproduced with attribution from: Life Saving Victoria. (2025). *Victorian Drowning Report 2024–25*. Life Saving Victoria: Melbourne. Royal Life Saving Australia (2026). *National Drowning Report 2026*. RLSSA: Sydney. Surf Life Saving Australia (2026). *Summer Coastal Drowning Report 2025/26*. SLSA: Sydney. These figures remain the property of the publishing organisations. If you adapt this resource, retain these citations.

Data sources cited in this resource. Life Saving Victoria. (2026). *Aquatic Injury Prevention Agenda 2026–27*. Life Saving Victoria: Melbourne. Life Saving Victoria. (2025). *Victorian Drowning Report 2024–25*. Life Saving Victoria: Melbourne. These figures remain the property of the publishing organisation. If you adapt this resource, retain these citations.